

All MCQs

1. *Regarding oropharyngeal functions the following answer is incorrect:
 - a. Conduit passage of air or food
 - b. Helps in pharyngeal phase of deglutition
 - c. Provide local defense and immunity against harmful intruders into the air and food passage
 - d. Forms part of the vocal tract for certain speech sounds
 - e. Helps in appreciation of smell
2. *The most common cause of bilateral recurrent laryngeal nerve paralysis is
 - a. Laryngeal trauma
 - b. Surgical iatrogenic damage in case of total thyroidectomy
 - c. A tumor mass expanding in upper part of mediastinum
 - d. Toxic polyneuropathy of recurrent laryngeal nerves
 - e. Acute subglottic laryngitis
3. *The most important symptom in otosclerosis is
 - a. Vertigo
 - b. Conductive hearing loss
 - c. Sensorineural hearing loss
 - d. Aural fullness
 - e. Tinnitus
4. *The election treatment in cholesteatoma is
 - a. Antibiotherapy
 - b. Chemotherapy
 - c. Radiotherapy
 - d. Surgery
 - e. Wait and scan policy
5. *The nasal septum divides the nose into
 - a. Superior and inferior nasal cavity
 - b. Right and left nasal cavity
 - c. Lateral and medial cavity
 - d. Four cavities 2 superior and 2 inferior
 - e. Four cavities 2 medial and 2 lateral
6. *Maxillary sinus
 - a. Opens into posterior part of the ethmoidal infundibulum into the middle meatus
 - b. Opens into the superior meatus
 - c. Its innervation is from the CN IX
 - d. It is the smallest sinus
 - e. Has a 5 ml capacity
7. Relations of external acoustic meatus are
 - a. Superiorly – middle cranial fossa
 - b. Posteriorly – mastoid air cell and facial nerve
 - c. Inferiorly – parotid gland
 - d. Anteriorly – temporo-mandibular joint
 - e. Superiorly – mastoid air cells and facial nerve
8. The Eustachian (pharyngotympanic) tube
 - a. Connects the tympanic cavity with the nasopharynx
 - b. Bony part – lat 1/3
 - c. Fibrocartilaginous part – medial 2/3
 - d. Torus tubaris at the pharyngeal end
 - e. Zygomatic cells in the middle part

9. Ossicles of the middle ear are
- Malleus
 - Tensor tympani
 - Incus
 - Stapes
 - Stapedius
10. Predisposing factors for acute suppurative otitis media are
- Recurrent attacks of common cold, upper respiratory tract infections and exanthematous fever
 - Infections of tonsils and adenoids
 - Chronic tympanosclerosis
 - Nasal allergy
 - Tumours of nasopharynx, packing of nose or nasopharynx for epistaxis
11. Signs of acute mastoiditis are
- Mastoid tenderness
 - Ear discharge: mucopurulent or purulent discharges
 - Sagging of inferior wall
 - Swelling over the mastoid
 - Hearing loss – conductive type
12. Otosclerosis symptoms are
- Hearing loss – bilateral neurosensorial type
 - Paracusis willisii – patient hears better in noisy environment than quiet surroundings
 - Tinnitus
 - Vertigo
 - Monotonous well modulated soft speech
13. Differential diagnosis of Otosclerosis
- Serous otitis media
 - Tympanosclerosis
 - Attic fixation of malleus head
 - Acoustic neuroma
 - Ossicular discontinuity
14. Meniere's disease cardinal symptoms are
- Episodic vertigo
 - Paracusis willisii
 - Fluctuating hearing loss
 - Tinnitus
 - Sense of fullness in involved ear
15. Nerve supply of the larynx
- Motor: recurrent laryngeal nerve
 - Sensory: internal laryngeal nerve (above vocal cord)
 - Motor: recurrent laryngeal nerve (below vocal cord)
 - Motor: recurrent laryngeal nerve (above vocal cord)
 - Sensory: recurrent laryngeal nerve (below vocal cord)
16. Chronic hyperaemic laryngitis treatment
- Elimination of the infection of upper or lower respiratory tract
 - Avoidance of irritating factors
 - Surgery
 - Voice rest and speech therapy
 - Expectorants

17. Congenital lesions of the larynx are
- Laryngomalacia
 - Subglottic haemangioma
 - Laryngoesophageal cleft
 - Pectus excavatum
 - Laryngeal web
18. Treatment of polypoid corditis (Reinke's edema)
- Smoking cessation
 - Surgery: cordectomy
 - Antireflux medication
 - Preoperative vocal therapy
 - Surgery: vocal cord stripping
19. Benign tumours of the larynx are
- Squamous cell carcinoma
 - Chondroma
 - Haemangioma
 - Granular cell tumour
 - Glandular tumour
20. Etiology of larynx cancer
- Tobacco, alcohol
 - Neck trauma
 - Previous radiation
 - Genetic factors
 - Occupational exposure
21. Glottic cancer symptoms are
- Hoarseness is an early symptom
 - Hoarseness is a late symptom
 - Stridor and laryngeal obstruction in late stages
 - Stridor and laryngeal obstruction in early stages
 - Throat pain
22. Types of cordectomies
- type I - a subepithelial cordectomy, which is resection of the epithelium;
 - type II - a subligamental cordectomy, which is a resection of the epithelium, Reinke's space and vocal ligament;
 - type III - transmuscular cordectomy, which proceeds through the vocalis muscle;
 - type IV - total cordectomy;
 - type Va - extended cordectomy, which encompasses the contralateral vocal fold and the anterior commissure;
 - type Vb - extended cordectomy, which includes the arytenoid;
 - type Vc - extended cordectomy, which encompasses the subglottis;
 - type Vd - extended cordectomy, which includes the ventricle.
23. Indications of tracheotomy are
- Respiratory obstruction
 - Retained secretions
 - Respiratory insufficiency
 - Puberophonia
 - Hoarseness
24. Types of tracheotomy are
- High tracheotomy; above the level of thyroid isthmus
 - High tracheotomy: through the II and III rings
 - Mid tracheotomy: through the II and III rings
 - Mid tracheotomy: below the level of thyroid isthmus
 - Low tracheotomy: below the level of thyroid isthmus

25. The frontal sinus drains
- In the superior meatus
 - Into frontal recess (55%)
 - Above but not into the infundibulum (30%)
 - Into the infundibulum (15%)
 - Above the bulla ethmoidalis
26. Functions of paranasal sinuses are
- Air conditioning of the inspired air: warming and humidifying
 - Voice resonance
 - Thermal insulators – protection of delicate structures in the orbit and cranium
 - Nasal drainage
 - Lighten the skull bones
27. Clinical features of acute maxillary sinusitis are
- Constitutional symptoms: fever, general malaise
 - Headache
 - Pain – over upper jaw, occasionally ipsilateral supraorbital region
 - Tenderness (pressure over the anterior wall of antrum produces pain)
 - Post nasal discharge (pus on the hard palate at anterior rhinoscopy)
28. Complications of acute maxillary sinusitis are
- Deep neck infection
 - Subacute, chronic sinusitis
 - Osteitis or osteomyelitis
 - Orbital cellulitis or abscess, optic neuritis
 - Cavernous sinus thrombosis
29. Clinical features of acute ethmoid sinusitis are
- Pain over the bridge of the nose, medial and deep to the eye; aggravated by movement
 - Edema of lids
 - Hoarseness
 - Nasal discharge
 - Swelling of the turbinate
30. Waldeyer's ring consists of
- Nasopharyngeal tonsil or the adenoids
 - Palatine tonsils
 - Lingual tonsils
 - Gastric tonsils
 - Mastoid tonsils
 - Tubal tonsils (in Fossa of Rosenmüller)
31. The hypopharynx clinically subdivides into three regions
- Pyriform sinus (fossa) lies on the either side of the larynx expands from pharyngoepiglottic fold ... end of esophagus
 - Postcricoid region: the part of the anterior wall of laryngopharynx between the upper and lower borders of cricoid lamina
 - Posterior pharyngeal wall: extends from the level of the hyoid bone to the level of cricoarytenoid
 - Vocal cord
 - Ventricular fold
32. *Functions of the hypopharynx are
- Conduit for passage of air or food
 - Breathing
 - Help in pharyngeal phase of deglutition
 - Air conditioning of the inspired air: warming and humidifying
 - Forms part of vocal tract for certain speech sounds

33. Peritonsillar abscess (Quinsy) complications are
- Parapharyngeal abscess
 - Larynx edema
 - Septicemia
 - Pneumonitis
 - Spontaneous hemorrhage from carotid vein or jugular artery
34. On the lateral wall of the nasal cavities we can find
- The middle meatus
 - The nasal septum
 - The inferior meatus
 - The maxillary sinus ostium
 - Adenoids
35. Nasal septum deviation can lead to
- Sinusitis
 - Oral respiration
 - Hypo-/anoxemia
36. * Regarding pharynx anatomy which of the following statements is incorrect:
- It extends from base of skull to inferior border of cricoid cartilage anteriorly and inf border of C6 posterior
- Widest portion (5cm) is at hyoid level
 - Narrowest portion (1.5cm) is at its caudal end
 - The pharynx is divided into 3 parts: naso-/ oro- and laryngopharynx
 - The pharynx has more than 20cm in length
37. The structures of pharyngeal wall are the following, with one exception. Indicate the exception:
- Mucous membrane
 - Pharyngeal aponeurosis
 - Muscular fascia
 - Buccopharyngeal fascia
 - Periosteum
38. Waldeyer's ring is composed of the following elements except one
- Submaxillary glands
 - Nasopharyngeal tonsil or the adenoids
 - Palatine tonsils
 - Tubal tonsils (in Fossa of Rosenmüller)
 - Nodules (on post pharyngeal wall)
39. *Nasopharyngeal functions are following except one
- Conduit for air
 - Ventilates the middle ear through Eustachian tube
 - Resonance
 - Passage for foods and liquids
 - Drainage channel
40. Adenitis aural symptoms are
- Tubal obstruction
 - Sinusitis
 - Recurrent attacks of acute otitis media
 - Nasal discharge
 - Serous otitis media
41. Regarding Juvenile Nasopharyngeal Angiofibroma the following statements are correct
- Exact cause unknown
 - Age and sex (exclusively in male in age group 40-60)
 - Appears as progressive nasal obstruction and nasal speech
 - Biopsy is associated with profound bleeding and therefore to be avoided
 - Treatment is always surgical
42. Regarding nasopharyngeal cancer following statements are correct

- a. Age onset: 5th to 7th decades but may involve younger age groups
 - b. Cervical nodal metastasis may be the only manifestation
 - c. Otologic symptoms and sign are represented by conductive hearing loss, serous and suppurative otitis media
 - d. Ophthmo-neurological symptoms are always present at the beginning
 - e. Distant metastasis are into the eustachian tube
43. Which of the following are symptoms of acute tonsillitis
- a. Sore throat
 - b. Difficulty in swallowing
 - c. Oral breathing
 - d. Fever
 - e. Ear ache
44. Which of the following are acute tonsillitis complications
- a. Convergent strabismus
 - b. Rheumatic fever
 - c. Chronic tonsillitis
 - d. Acute glomerulonephritis
 - e. Subacute bacterial endocarditis
45. DD of acute tonsillitis is performed with
- a. Membranous tonsillitis
 - b. Diphtheria
 - c. Rhinitis
 - d. Vincent's angina
 - e. Infections mononucleosis
46. DD of acute tonsillitis is performed with
- a. Sinusitiy
 - b. Buccopharyngeal manifestations in agranulocytosis
 - c. Buccopharyngeal manifestations of leukaemia
 - d. Aphthous ulcers
 - e. Malignant tumour of the tonsil
47. Regarding hypopharyngeal malignant tumour which of the following statements are correct
- a. Clinical features are represented by progressive dysphagia, malnutrition, weight loss, change of voice and dysphonia
 - b. Mirror examination may not be useful
 - c. The most frequent site of origin is subglottic region
 - d. Prognosis is poor
 - e. Surgery, radiotherapy and combined therapy
48. *The olfactory nerves (12-20) which pass through cribriform plate and end in the
- a. Frontal lobe
 - b. Temporal lobe
 - c. Olfactory lobe
 - d. Parietal lobe
 - e. Olfactory bulb
49. The rinophyma is a
- a. Benign slow growing mass
 - b. Slow growing malignant tumour
 - c. Appears to the hypertrophy of the sebaceous glands of the tip of the nose
 - d. Is treated with surgical excision or with laser Co2
 - e. Appears due to the hypertrophy of the perspiratory glands of the tip of the nose

50. The choanal atresia
- Can be uni-, bilateral, complete, incomplete, bony (90%) or membranous (10%)
 - It is always bilateral, complete, bony or membranous
 - It is an emergency in bilateral choanal atresia
 - One of the diagnostic signs is the absence of air bubbles in nasal discharge
 - An important diagnosis sign is the failure to pass a catheter from nose to pharynx
51. *A dermoid cyst is
- A malignant tumour
 - Congenital tumour
 - Infection
 - Blood collection
 - Sign of consumption
52. *Into the middle meatus open the following sinuses
- Maxillary, frontal and anterior ethmoidal
 - Posterior ethmoidal sinuses
 - Sphenoid sinus
 - Cavernous sinus
 - Sinus tympani
53. Septal haematoma
- Is a collection of blood under the perichondrium or periosteum of the nasal septum
 - It can be caused by nasal trauma or septal surgery
 - Treatment consists in aspiration with a wide bore sterile needle
 - Incision and drainage in larger ones with bilateral nasal packing
 - Is a collection of connective tissue
54. Nasal skin, which affirmations are correct
- Thin skin and freely mobile, covers the nasal bones
 - Thin skin with many sebaceous glands, covers nasal bone and lower lateral cartilages
 - Thick adherent and rich in sebaceous glands skin cover the alar cartilages
 - Thin and adherent skin covering the alar cartilages
 - Thick skin with many sebaceous glands, covers nasal bone and upper lateral cartilages
55. Rhinoscleroma
- Etiology is a gram-negative bacillus – *Klebsiella Rhinoscleromatis*
 - Etiology is a gram-positive – *Staph aureus*
 - Clinical stages are: atrophic, granulomatous, cicatricial
 - Certain diagnosis given by biopsy
 - Treated with streptomycin and tetracycline
56. Treatment in allergic rhinitis the following are correct
- Avoidance of allergen
 - Supraexposure to allergen
 - Antihistaminics
 - Corticosteroids (oral and topical)
 - Radiotherapy
57. Symptoms of vasomotor rhinitis are
- Wheezing
 - Excessive rhinorrhea
 - Nasal obstruction
 - Postnasal drip
 - Cough

58. Local causes of epistaxis are
- Trauma / foreign bodies
 - Infections
 - Neoplasms of nose and paranasal sinuses
 - Heat
 - Cold
59. Posterior epistaxis
- Originates mostly from posterosuperior part of nasal cavity
 - Occurs spontaneous often due to hypertension or arteriosclerosis
 - Most common cause is trauma
 - Mostly occurs in children or young adults
 - Mild and easy to control
60. Complications of frontal sinusitis
- Orbital cellulitis
 - Osteomyelitis of frontal bone
 - Meningitis, extradural abscess or frontal lobe abscess
 - Maxillary sinus necrosis
 - Acute conjunctivitis
61. Correct affirmation about laryngomalacia
- Most common congenital laryngeal anomaly (50-75%)
 - Most frequent cause of stridor in adults
 - Male predominance 2:1
 - Flaccidity of supraglottic laryngeal tissue
 - Characterised by inward collapse of pharynx structures
62. Subglottic haemangiomas are
- Asymptomatic till 12 months
 - 50% have associated cutaneous haemangiomas
 - Stridor but a normal cry
 - Reddish-blue mass below the vocal cords
 - Biopsy – strongly indicated
63. *Epiglottis is formed by
- Muscle
 - Bone
 - Fibrous cartilage
 - Adipose tissue
 - Skin
64. *Most common cause of bilateral recurrent laryngeal nerve paralysis is
- Laryngeal trauma
 - Surgical iatrogenic damage in case of total thyroidectomy due to thyroid gland carcinoma
 - Tumour mass expanding in upper part of mediastinum
 - Toxic polyneuropathy of recurrent laryngeal nerves
 - Acute subglottic laryngitis
65. Larynx functions are
- Phonation
 - Respiration
 - Inferior respiratory airway protection
 - thoracic fixation
 - Expectoration

66. Following are intracranial lesion of otitis media

- a. Extradural abscess
- b. Petrositis
- c. Meningitis
- d. Labyrinthitis
- e. Brain abscess

67. Abscesses in relation to mastoid infection are

- a. Postauricular abscess
- b. Zygomatic abscess
- c. Bezold abscess
- d. Petrositis
- e. Orbital cellulitis

68. Adenoiditis aural symptoms are not:

- a. Tubal obstruction
- b. Sinusitis
- c. Recurrent attacks of acute otitis media
- d. Nasal discharge
- e. Serous otitis media

69. *The epiglottis is formed by

- a. Muscle
- b. Bone
- c. Fibrous cartilage
- d. Adipose tissue
- e. Skin

70. Which of the following statements are **correct**:

- a. In Weber test the tuning fork is placed on the mastoid process
- b. In Weber test a patient with normal hearing localizes the tone either in the center of the head or equally in both ears
- c. In Weber test a patient with unilateral conductive hearing loss localizes the tone in the affected ear
- d. In Weber test a patient with neurosensorial hearing loss localizes the tone in the affected ear
- e. In Weber test a patient with neurosensorial hearing loss localizes the tone in the healthy ear

71. Differential diagnosis of acute tonsillitis is performed with

- a. Aphthous ulcers
- b. Malignant tumor of the tonsil
- c. Sinusitis
- d. Bucopharyngeal manifestations in Agranulocytosis
- e. Bucopharyngeal manifestations in Leukemia

72. Otosclerosis is characterized by

- a. Eustachian tube function is impaired
- b. Tinnitus
- c. Hyalinisation and calcification of subepithelial connective tissue, white chalky deposits in the promontory and ossicles
- d. Conductive hearing loss
- e. Paracusis willisii

73. Chronic non specific laryngitis can NOT be:

- a. Catarrhal
- b. Caused by tbc
- c. Hypertrophic red and white type
- d. Atrophic
- e. Caused by syphilis

74. Waldeyer's ring is composed of the following elements except one

- a. Submaxillary glands
 - b. Nasopharyngeal tonsil or the adenoids
 - c. Palatine tonsils
 - d. Tubal tonsils (in Fossa of Rosenmüller)
 - e. Nodules (on post pharyngeal wall)
75. Waldeyer's ring is NOT composed of the following elements:
- a. Nasopharyngeal tonsil or the adenoids
 - b. Palatine tonsils
 - c. Lingual tonsils
 - d. Gastric tonsils
 - e. Mastoid tonsils
 - f. Tubal tonsils (in Fossa of Rosenmüller)
76. In acute mastoiditis
- a. Treatment consists of stapedotomy
 - b. Treatment consists of antibiotics
 - c. Treatment consists of reconstructive surgery with hearing restoration by tympanoplasty
 - d. Treatment consists of myringotomy
 - e. Treatment consists of mastoidectomy
77. Abscesses in relation to mastoid infection are:
- a. Bezold
 - b. Petrositis
 - c. Zygomatic
 - d. Orbital cellulitis
 - e. Postauricular
78. Symptoms of acute tonsillitis are
- a. Oral breathing
 - b. Difficulty in swallowing
 - c. Fever
 - d. Sore throat
 - e. Earache
79. The hypopharynx is clinically subdivided into
- a. Vocal cord
 - b. Ventricular fold
 - c. Pyriform sinus
 - d. Post-Cricoid area
 - e. Posterior pharyngeal wall
80. Larynx benign tumors are
- a. Chondroma
 - b. Osteoma
 - c. Haemangioma
 - d. Botriomycoma
 - e. Papilloma
81. Congenital lesions of the larynx are
- a. Laryngomalacia
 - b. Choanal atresia
 - c. Laryngeal web
 - d. Cleft palate
 - e. Laryngocele
82. * The maxillary sinus
- a. Opens into the superior meatus
 - b. Its innervation is from the IXth cranial nerve
 - c. It is the largest sinus
 - d. Opens into the superior meatus

- e. Has a 5ml capacity
83. Symptoms of vasomotor rhinitis are NOT
- a. Nasal obstruction
 - b. Bronchospasm
 - c. Postnasal drip
 - d. Wheezing
 - e. Ocular prurit
84. Vocal nodules
- a. Are considered tumoral tissue reactions
 - b. More frequent in male patients
 - c. Are not favoured by voice overload
 - d. Are surgically treated
 - e. Voice rest is recommended
85. Complications of laryngotracheal trauma are represented by:
- a. Loss of taste
 - b. Perichondritis/ laryngeal abscess
 - c. Vocal cord paralysis
 - d. Laryngeal stenosis
 - e. Hiposmia
86. Laryngotracheal trauma can be represented by
- a. Haematoma and edema
 - b. Othematoma
 - c. Dislocation of cricoarytenoid joint
 - d. Fractures of hyoid bones, thyroid and cricoids cartilages
 - e. Fractures of upper tracheal rings
87. Following are intratemporal complications of otitis media
- a. Subdural abscess
 - b. Facial paralysis
 - c. Mastoiditis
 - d. Meningitis
 - e. Petrositis
88. The structures of oropharyngeal wall are the following with one exception
- a. Mucous membrane
 - b. Pharyngeal aponeurosis
 - c. Muscular fascia
 - d. Buccopharyngeal fascia
 - e. Periosteum